

Media Information

5:08 PM

Connect - Test Details

SURGICAL PATHOLOGY

Collected on Sep 08, 2025 4:05 PM

Important Information About Your Test Results



Results

Surgical Pathology (AP)**Value**

New York-Presbyterian Hospital/Weill Cornell Medical Center
Department of Pathology Surgical Pathology Laboratory
Starr 1028
525 East 68th Street, NY, NY

Surgical Pathology Report

Specimen Date:
In Lab Date: 18:13
Report Date:

Patient:
60 E 8TH ST APT 16L Cornell Medical Associates
APT 16L 12 West 72nd Street
New York, NY
Telephone: (646) 962-7800 Fax: 962-0413

Encounter #: 315706558
Location: WCINYP US 61ST
Age/Sex: 81 / F
Copy To: Jill S. Gluskin, MD

CLINICAL INFORMATION:

Abnormal finding on breast imaging (R92.8). Left breast 3:00 axis 6 cm FN 1.3 cm mass, BI-RADS 4C.

DIAGNOSIS:

A. Breast, left, 3:00 N6, ultrasound guided core biopsy:
Invasive ductal carcinoma and atypical lobular hyperplasia (ALH).
The invasive carcinoma is moderately differentiated,
i.e., grade 2: 3+2+1,
displays lobular features,
and involves multiple cores,
wherein it spans ~0.8 cm.
Neither lymphovascular involvement nor in situ carcinoma is identified.
A brisk lymphoplasmacytic infiltrate is absent (<50%).
Results of ER, PR, HER2, and Ki67 tests, to be performed on 'A1' sections, will follow

<https://www.myconnectnyc.org/mychart/app/test-results/details?pageMode=1&eorderid=WP-24CaeD-2BHGjXGzt4FBGut-2Fw-2Fw-3D-3D-24gwr1UjTN...> 3/9

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Show

E-cadherin immunoreactivity of the invasive carcinoma, also on 'A1', supports the ductal phenotype of the invasive carcinoma.

E-cadherin immunonegativity of the lesional cells ('A1') supports the diagnosis of ALH.

Rebecca Filorent/gea905

GROSS DESCRIPTION:

A. Received in formalin, the specimen is labeled "ultrasound-guided core biopsy left breast 3:00 N6" and consists of four cores of yellow-white tissue measuring from 0.8 x 0.1 x 0.1 cm to 1.5 x 0.1 x 0.1 cm. Also received are fragments of yellow-white soft

tissue measuring 0.4 x 0.4 x 0.1 cm in aggregate. The specimen was placed in formalin at 4:36 p.m. and placed into the tissue processor at 4 o'clock. 100% submitted. Summary of sections: A1-A3. (RLK/REF)

**** Electronically Signed Out ****

Andy Phan, MD

Attending

The attending
the diagnostic slides, and has edited the gross and microscopic portions of the report in rendering the final diagnosis.

ICD Codes: A; C50.812 Z17.0 Z17.21

Billing Codes: A; 23883051, 23883421, 23883601(4) (Epic Outpatient Billing)

SNOMED:

AVP900/lpm

PROCEDURES/ADDENDA:

Specimen: Left breast (invasive carcinoma) Block: A1

RESULTS OF ANCILLARY IMMUNOPATHOLOGY STUDIES

PERCENTAGE OF POSITIVE CELLS STAINING INTENSITY

ESTROGEN RECEPTOR: >95%

STRONG

<https://www.myconnectnyc.org/mychart/app/test-results/details?pageMode=1&eorderid=WP-24CaeD-2BHGjXGzt4FBGut-2Fw-2Fw-3D-3D-24gwr1JTN...> 4/9

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PROGESTERONE RECEPTOR: ~90% STRONG**PROLIFERATION INDEX (Ki-67): 20%****HER2: 0**

INTERPRETATION: Since >1% of tumor cell nuclei express estrogen receptors, this tumor is considered to be **ESTROGEN RECEPTOR POSITIVE**. Since >1% of tumor cell nuclei express progesterone receptors, this tumor is considered to be **PROGESTERONE RECEPTOR POSITIVE**. HER2: no [REDACTED] [REDACTED] [REDACTED] result indicates **NEGATIVE HER2** by immunohistochemistry.

This test was developed and its performance characteristics determined by the Immunopathology Laboratory. It has not been cleared or approved by the U.S. Food and Drug Administration (FDA). The FDA has determined that such clearance or approval is not necessary. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high complexity clinical laboratory testing. Antibodies and methodology: ER (Ventana, SP1); PR (Leica, 16); Anti-HER2/neu (4B5) Rabbit Monoclonal Primary Antibody (Ventana/Roche)/Ultraview Universal DAB Detection Kit; Ki-67 (Dako MIB-1). The interpretation criteria are in accordance with the guidelines of the American Society of Clinical Oncology/College of American Pathologists Guideline Recommendations for Immunohistochemical Testing of Estrogen and Progesterone Receptors in Breast Cancer by American Society of Clinical Oncology (2010 and 2020) and American Society of Clinical Oncology/College of American Pathologists Guideline and Focused Update Recommendations for Human Epidermal Growth Factor Receptor 2 Testing in Breast Cancer in Journal of Clinical Oncology (2007, 2013, and 2018). Duration of fixation in neutral buffered formalin is between 6 and 72 hours. This assay has not been validated decalcified tissues. Results should be interpreted with caution given the likelihood of false negativity on decalcified specimens.

**** Electronically Signed Out By ****

Andy Phan, MD

Attending [REDACTED]

The attending [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED]
the diagnostic slides,

and has edited the gross and microscopic portions of the report in rendering the

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final diagnosis.

All control slides for immunohistochemical tests show appropriate immunoreactivity. Immunohistochemical, in-situ hybridization and flow cytometric assays were developed and their performance characteristics determined by the Immunopathology Laboratory. The limit of enumeration for flow cytometry minimal residual diagnosis (MRD) detection assays is 0.01% of viable cells. These tests have not been cleared or approved by the U.S. Food and Drug Administration (FDA). The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes and should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high complexity clinical laboratory testing. The testing was performed at New York Presbyterian - Weill Cornell Medical Center, 525 East 68th Street, New York, NY [REDACTED] CLIA # 33D0653378.

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Connect - Test Details

 Scan 1

Ordering provider: 
Collection date: Sep 08, 2025 4:05 PM
Specimens: AP Specimen (Surgical)
Result date: Sep 12, 2025 2:14 PM
Result status: Final (Edited)
Resulting lab:
CORNELL PATHOLOGY

Document Information

Other: Outside Pathology

PATH

00:00

Attached To:

OUTSIDE OTHER ORDERS

Orders Only on with Historical Provider,

Source Information

Waynasia Memminger | Msk Pas